

A Practical Guide to Theatre-to-ICU Obstetric Handover

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Abstract

Handover between theatre and the intensive care unit is a recognised point of risk, particularly for obstetric patients who may have ongoing haemorrhage risk, pre-eclampsia, or cardiovascular instability. This guide sets out a structured, repeatable handover sequence intended for use by residents rotating between obstetric theatre and the ICU, drawing on the wider patient-safety-checklist literature.

Keywords: handover, obstetric anaesthesia, patient safety, ICU admission

Departmental guide

Prepared by the Residents' Academic Committee

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Summary. Handover between theatre and the intensive care unit is a recognised point of risk, particularly for obstetric patients who may have ongoing haemorrhage risk, pre-eclampsia, or cardiovascular instability. This guide sets out a structured, repeatable handover sequence intended for use by residents rotating between obstetric theatre and the ICU.

KEYWORDS · HANDOVER · OBSTETRIC ANAESTHESIA · PATIENT SAFETY · ICU ADMISSION

1. Why a structured handover matters

Verbal handovers that aren't structured tend to omit items unpredictably — often exactly the detail that matters for the next few hours of care. A short,

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consistent structure reduces that risk without adding much time to a busy transfer. This mirrors the logic behind the WHO Surgical Safety Checklist, which was shown to reduce post-operative morbidity and mortality across a range of hospital settings when used consistently [1, 2].

2. Suggested handover structure

1. **Patient identification & background** — name, age, parity, key obstetric history, indication for surgery.
2. **Procedure performed** — type of anaesthesia, surgical procedure, any intra-operative complications.
3. **Haemodynamic course** — blood loss, fluids and blood products given, vasopressor use, any instability and how it was managed.
4. **Airway and respiratory status** — extubation status, oxygen requirement, any airway concerns for the ICU team to be aware of.
5. **Analgesia plan** — what has been given, what is prescribed next, and any regional technique in place (e.g. spinal opioid, wound catheter).
6. **Outstanding concerns and thresholds for escalation** — specifically what should prompt the ICU team to call anaesthesia back.

3. Using this guide

This is a teaching resource for residents, not a substitute for local protocol. Departments should adapt the sequence to match their own documentation and handover tools, and residents should confirm current local practice with their supervising consultant.

Have a guide, protocol summary, or teaching resource to contribute? Email the department to have it added here.

References

- [1] A. B. Haynes, T. G. Weiser, W. R. Berry, S. R. Lipsitz, A.-H. S. Breizat, E. P. Dellinger, T. Herbosa, S. Joseph, P. L. Kibatala, M. C. M. Lapitan, A. F. Merry, K. Moorthy, R. K. Reznick, B. Taylor, A. A. Gawande, A surgical safety checklist to reduce morbidity and mortality in a global population, *New England Journal of Medicine* 360 (5) (2009) 491–499. [doi:10.1056/NEJMs0810119](https://doi.org/10.1056/NEJMs0810119).
- [2] Safe Surgery Saves Lives Study Group, [WHO Guidelines for Safe Surgery 2009: Safe Surgery Saves Lives](https://www.who.int/publications/i/item/9789241598552), World Health Organization, Geneva, 2009. URL <https://www.who.int/publications/i/item/9789241598552>